

WEAKNESS OR TIREDNESS

Give urgent attention to the patient with weakness or tiredness and any of:

- New sudden asymmetric weakness or numbness of face, arm or leg; difficulty speaking or visual disturbance: consider **stroke** or **TIA** → 136
- Chest pain → 37
- Difficulty breathing or respiratory rate ≥ 30 → 38
- Difficulty breathing worse on lying flat and leg swelling, **heart failure** likely → 137.
- Temperature $\geq 38^\circ\text{C}$ now or in past few days → 24
- Glucose < 3 (or < 4 if diabetes)
- Glucose ≥ 11.1
- Dehydration: thirst, dry mouth, poor skin turgor, drowsiness/confusion, BP $< 90/60$, pulse ≥ 100
- Hb < 6 → 27
- Worsening weakness of leg/s

Management:

- If dehydrated, give **oral rehydration solution (ORS)** and observe. If unable to drink or BP $< 90/60$, give **sodium chloride 0.9%** 500mL IV over 30 minutes, repeat until systolic BP > 90 . Continue 1L 6 hourly. Stop if breathing worsens. If IV rehydration needed or no better with oral rehydration after 2 hours, refer.
- If glucose < 3 or ≥ 11.1 → 17 or if diabetes and glucose < 4 → 130.
- If worsening weakness of leg/s, refer urgently.

Approach to patient with tiredness not needing urgent attention:

- Look for a cause for tiredness when it persists so that the patient is unable to complete routine tasks and it disrupts work, social and family life.
- First check symptoms, medications, mental health and for chronic conditions:

Check symptoms

- If fever now or in past 3 days → 24.
- If cough, weight loss, night sweats or fever, exclude TB → 92.
- If difficulty breathing worse on lying flat and leg swelling, **heart failure** likely → 135.
- If patient has difficulty sleeping → 87.
- If weight gain, low mood, dry skin, constipation or cold intolerance, check TSH. If abnormal, refer to doctor.

Check chronic conditions

- Test for HIV → 110. If HIV positive, give routine care → 111.
- Exclude pregnancy → 157.
- Exclude anaemia: check Hb. If < 12 (woman) or < 13 (man), **anaemia** likely → 27.
- Exclude diabetes: check glucose → 17.
- If ongoing symptoms following acute COVID-19, assess for Long COVID → 42.
- If patient has a life-limiting illness, also consider giving palliative care → 170.

Check medications

- If on abacavir or zidovudine, check for urgent side effects → 116.
- Chlorphenamine, enalapril, amlodipine, fluoxetine, amitriptyline, metoclopramide, sodium valproate, phenytoin and spironolactone can cause weakness or tiredness. Discuss with doctor.

Check mental health

- In the past month, has patient: 1) felt down, depressed, hopeless or 2) felt little interest or pleasure in doing things? If yes to either → 143.
- In the past year, has patient: 1) drunk ≥ 4 drinks¹/session, 2) used illegal drugs or 3) misused prescription or over-the-counter medications? If yes to any → 142.
- If none of the above, assess for stress and anxiety → 86.

Is there muscle weakness on examination²?

No

Ask about duration of tiredness:

< 1 month

- If any other symptoms managed above, reassure that tiredness should resolve with treatment.
- Advise to return if no better in 1 month.

≥ 1 month

- Check FBC, differential count, sodium, calcium, creatinine and ALT. If abnormal, discuss with doctor.
- If likely cause found, reassure that tiredness should resolve with treatment. If tiredness persists despite treatment, discuss/refer.
- If no cause found, review in 1 month. If tiredness persists and cause still not found, discuss/refer.

Yes

- If available, refer to doctor. Doctor to confirm weakness and:
 - Check potassium, sodium, calcium, phosphate, TSH.
 - Discuss/refer to specialist.
- If no doctor available, refer.

¹One drink is 1 tot of spirits, or 1 small glass (125mL) of wine or 1 can/bottle (330mL) of beer. ²If unsure how to examine for muscle weakness, discuss with doctor.